Kestrel Bay Physiotherapy

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Initial Assessment - Musculoskeletal

Client: Mitchell, Sarah L. Client ID: KBP-10427 DOB: 22 Sep 1983

Date of assessment: 03/03/2026 Clinician: Tom Reyes, PT, DPT
Referral: self-referral (direct access). Family doctor: Dr. Alan Pierce

Subjective

42-year-old graphic designer, right-hand dominant. Fell from her bicycle on 02/14/2026 onto an outstretched right arm. Seen at urgent care the same day; x-ray showed no fracture or dislocation. Pain over the lateral right shoulder, 6/10 at worst and 2/10 at rest. Aggravated by overhead reaching, putting on a coat and lying on the right side. Wakes two to three times a night. Goals: return to her cycling commute and to full days at the computer without pain.

Past history: left ankle sprain 2023, fully recovered. Mild asthma. No red flags reported (no weight loss, no night sweats, no neurological symptoms).

Objective

Posture: mildly protracted right scapula. AROM right shoulder: flexion 120 deg, abduction 95 deg with a painful arc from 70 to 110 deg, external rotation 45 deg, internal rotation to L3. Left shoulder full range.

Strength: right external rotation 4-/5, abduction 4/5, internal rotation 5/5. Special tests: Hawkins-Kennedy positive, empty can positive for pain, drop arm negative, external rotation lag sign negative. Cervical screen clear.

Assessment

Post-traumatic right supraspinatus tendinopathy with secondary scapular dyskinesis. No clinical signs of a full-thickness rotator cuff tear.

Plan

Twice weekly for six weeks, then review. Manual therapy, isometric loading progressing to isotonic rotator cuff and scapular strengthening. Home exercise programme issued. Invite to the Tuesday shoulder strength group from week six. Re-assess at four weeks; refer back to the family doctor for ultrasound if there is no improvement.

Electronically signed: Tom Reyes, PT, DPT 03/03/2026 11:42